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MC-2977

PLEASE SCAN QR CODE
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Name	: **MRS.YAMUNA KALYANI NEPPALA**	TID/SID	: UMR3504977/ 30363136
Age / Gender	: 36 Years / Female	Registered on	: 20-Sep-2025 / 07:58 AM
Ref.By	: -	Collected on	: 20-Sep-2025 / 09:04 AM
Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 17:12 PM
Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY II

25-Hydroxy Vitamin D

Investigation	Observed Value	Biological Reference Interval
25 Hydroxy Vitamin D Method:ECLIA	**14.4**	Deficiency: < 20 ng/mL Insufficiency: 20 - 30 ng/mL Sufficiency: 30 - 100 ng/mL Toxicity: >100 ng/mL **Note:** Biological Reference Ranges are changed due to change in method of testing.
Note	Kindly correlate clinically	

Interpretation:
1.Vitamin D is a family of compounds that is essential for the proper growth and formation of teeth and bones. This test measures the level of vitamin D in the blood.
2.Two forms of vitamin D can be measured in the blood, 25-hydroxyvitamin D and 1,25-dihydroxyvitamin D. The 25-hydroxyvitamin D is the major form found in the blood and is the relatively inactive precursor to the active hormone, 1,25-dihydroxyvitamin D. Because of its long half-life and higher concentration, 25-hydroxyvitamin D is commonly measured to assess and monitor vitamin D status in individuals.
3. The main role of vitamin D is to help regulate blood levels of calcium, phosphorus, and (to a lesser extent) magnesium.
4 Vitamin D is vital for the growth and health of bone; without it, bones will be soft, malformed, and unable to repair themselves normally, resulting in diseases called rickets in children and osteomalacia in adults.
5. Vitamin D has also been shown to influence the growth and differentiation of many other tissues and to help regulate the immune system. These other functions have implicated vitamin D in other disorders, such as autoimmunity and cancer.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

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Dr Afreen Anwar
Consultant Biochemist

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Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY II

Thyroid Profile (T3,T4,TSH)

Investigation	Observed Value	Biological Reference Interval
Triiodothyronine Total (T3) Method:ECLIA	1.29	0.80-2.00 ng/mL Pregnancy: 1st Trimester: 0.81 - 1.90 ng/mL 2nd & 3rd Trimester: 1.00 - 2.60 ng/mL
Thyroxine Total (T4) Method:ECLIA	10.1	5.1-14.1 µg/dL
Thyroid Stimulating Hormone (TSH) Method:ECLIA	1.56	0.27-4.20 µIU/mL Pregnancy: 1st Trimester: 0.1 - 2.5 µIU/mL 2nd Trimester: 0.2 - 3.0 µIU/mL 3rd Trimester: 0.3 - 3.0 µIU/mL

Interpretation:

A thyroid profile is used to evaluate thyroid function and/or help diagnose hypothyroidism and hyperthyroidism due to various thyroid disorders. T4 and T3 are hormones produced by the thyroid gland. They help control the rate at which the body uses energy, and are regulated by a feedback system. TSH from the pituitary gland stimulates the production and release of T4 (primarily) and T3 by the thyroid. Most of the T4 and T3 circulate in the blood bound to protein. A small percentage is free (not bound) and is the biologically active form of the hormones.
Reference: Tietz textbook of Clinial Chemistry and Molecular Diagnostics, Nader Rifia, Andrea Ritas Horvath, Carl T. Wittwer.

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Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 14:19 PM
Sample Type	: Urine	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL PATHOLOGY

Complete Urine Examination (CUE)

Investigation	Result	Biological Reference Intervals
Physical Examination		
Colour Method:Physical	Pale yellow	Straw to Yellow
Appearance Method:Physical	Clear	Clear
Chemical Examination		
Reaction and pH Method:Indicator	Acidic (6.5)	4.6-8.0
Specific gravity Method:Refractometry	1.003	1.000-1.035
Protein Method:Protein Error of pH indicators	Negative	Negative
Glucose Method:Glucose oxidase/Peroxidase	Negative	Negative
Blood Method:Peroxidase	Negative	Negative
Ketones Method:Sodium Nitroprusside Method	Negative	Negative
Bilirubin Method:Diazonium salt	Negative	Negative
Leucocytes Method:Esterase reaction	Negative	Negative
Nitrites Method:Modified Griess reaction	Negative	Negative
Urobilinogen Method:Diazonium salt	Negative	Up to 1.0 mg/dl (Negative)
Microscopic Examination		
Pus cells (leukocytes) Method:Flow Digital Imaging/Microscopy	1-2	2 - 3 /hpf
Epithelial cells Method:Flow Digital Imaging/Microscopy	1-2	2 - 5 /hpf
RBC (erythrocytes) Method:Flow Digital Imaging/Microscopy	Absent	Absent
Casts Method:Flow Digital Imaging/Microscopy	Absent	Occasional hyaline casts may be seen

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TEST REPORT

Crystals Method:Flow Digital Imaging/Microscopy	Absent	Phosphate, oxalate, or urate crystals may be seen
Others Method:Flow Digital Imaging/Microscopy	Nil	Nil

Method: Automated

Reference: Godkar Clinical Diagnosis and Management by Laboratory Methods, First South Asia edition. Product kit literature.

Interpretation:

The complete urinalysis provides a number of measurements which look for abnormalities in the urine. Abnormal results from this test can be indicative of a number of conditions including kidney disease, urinary tract infecation or elevated levels of substances which the body is trying to remove through the urine . A urinalysis test can help identify potential health problems even when a person is asymptomatic. All the abnormal results are to be correlated clinically.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Ayeesha

Dr Shaikh Ayeesha
Consultant Hematopathologist
Reg.No - TSMC/FMR/00158

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Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 13:03 PM
Sample Type	: Whole Blood	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF HEMATOPATHOLOGY

Erythrocyte Sedimentation Rate (ESR)

Investigation	Observed Value	Biological Reference Intervals
ESR 1st Hour Method:Westergren/Vesmatic	**25**	<=12 mm/hour

Complete Blood Count (CBC)

Investigation	Observed Value	Biological Reference Intervals
Hemoglobin Method:Spectrophotometry	**9.6**	12.0-15.0 g/dL
PCV/HCT Method:Calculated	**29.2**	36.0-46.0 vol%
Total RBC Count Method:Electrical Impedance	4.40	3.80-4.80 mill /cu.mm
MCV Method:Calculated	**66.3**	83.0-101.0 fL
MCH Method:Calculated	**21.7**	27.0-32.0 pg
MCHC Method:Calculated	32.8	31.5-34.5 g/dL
RDW (CV) Method:Calculated	**18.1**	11.6-14.0 %
MPV Method:Calculated	8.7	7.0-10.0 fL
Total WBC Count Method:Electrical Impedance	5570	4000-10000 cells/cumm
Platelet Count Method:Electrical Impedance	3.16	1.50-4.10 lakhs/cumm
Differential count		
Neutrophils Method:Microscopy	62.9	40.0-80.0 %
Lymphocytes Method:Microscopy	28.2	20.0-40.0 %
Eosinophils	1.3	1.0-6.0 %
Monocytes	7.1	2.0-10.0 %
Basophils Method:Flowcytometry/Electrical Impedance/Microscopy	**0.5**	< 1.0-2.0 %

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TEST REPORT

Absolute Neutrophil Count Method:Calculated	3504	2000-7000 cells/cumm
Absolute Lymphocyte Count (ALC)	1571	1000-3000 cells/cumm
Absolute Eosinophil Count (AEC)	72	20-500 cells/cumm
Absolute Monocyte Count Method:Calculated	395	200-1000 cells/cumm
Absolute Basophil Count Method:Calculated	28	20-100 cells/cumm
Neutrophil - Lymphocyte Ratio(NLR) Method:Calculated	2.23	0.78-3.53

Method: Automated Hematology Cell Counter, Microscopy

Reference: Dacie and Lewis Practical Hematology,12th Edition.
Wallach's interpretation of diagnostic tests, Soth Asian Edition.

Interpretation: A Complete Blood Picture (CBP) is a screening test which can aid in the diagnosis of a variety of conditions and diseases such as anemia, leukemia, bleeding disorders and infections. This test is also useful in monitoring a person's reaction to treatment when a condition which affects blood cells has been diagnosed. All the abnormal results are to be correlated clinically.

Note: These results are generated by a fully automated hematology analyzer and the differential count is computed from a total of several thousands of cells. Therefore the differential count appears in decimalised numbers and may not add upto exactly 100. It may fall between 99 and 101.

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Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 13:17 PM
Sample Type	:	Reference	: HCL Avitas Pvt Ltd

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TEST REPORT

DEPARTMENT OF CARDIOLOGY

Physical Examination (BP, HT, WT, BMI)

Investigation	Observed Value	
BP	100/80	
Weight	61	Kg
Height	160	cm
BMI	23.83	

Ground Floor, Q-Mart Building, Sreshta Marvel, Gachibowli

--- End Of Report ---

Doctor

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Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Blood Urea Nitrogen (BUN)

Investigation	Observed Value	Biological Reference Interval
Blood Urea Nitrogen. Method:Calculated	6.4	6-20 mg/dL

Interpretation: Urea is a waste product formed in the liver when protein is metabolized. Urea is released by the liver into the blood and is carried to the kidneys, where it is filtered out of the blood and released into the urine. Since this is a continuous process, there is usually a small but stable amount of urea nitrogen in the blood. However, when the kidneys cannot filter wastes out of the blood due to disease or damage, then the level of urea in the blood will rise. The blood urea nitrogen (BUN) evaluates kidney function in a wide range of circumstances, to diagnose kidney disease, and to monitor people with acute or chronic kidney dysfunction or failure. It also may be used to evaluate a person's general health status as well.

Reference: Tietz Fundamentals of Clinical Chemistry and Molecular Diagnostics

Calcium, Serum

Investigation	Observed Value	Biological Reference Interval
Calcium Method:BAPTA	8.9	8.6-10.0 mg/dL

Interpretation: Calcium is essential for bones, heart, nerves, kidneys, and teeth. Serum calcium levels are vital to detect hypocalcemia, hypercalcemia and associated disorders. Parathormone (PTH) and vitamin D are responsible for maintaining calcium concentrations in the blood within a narrow range of values. Serum calcium levels are diagnostic in cases of Kidney stones, Bone diseases and Neurologic disorders.

Creatinine, Serum

Investigation	Observed Value	Biological Reference Interval
Creatinine. Method:Alkaline Picrate	0.64	0.50-0.90 mg/dL

Interpretation:

Creatinine is a nitrogenous waste product produced by muscles from creatine. Creatinine is majorly filtered from the blood by the kidneys and released into the urine, so serum creatinine levels are usually a good indicator of kidney function. Serum creatinine is more specific and more sensitive indicator of renal function as compared to BUN because it is produced from muscle at a constant rate and its level in blood is not affected by protein catabolism or other exogenous products. It is also not reabsorbed and very little is secreted by tubules making it a reliable marker. Serum creatinine levels are increased in pre renal, renal and post renal azotemia, active acromegaly and gigantism. Decreased serum creatinine levels are seen in pregnancy and increasing age.

Glucose Fasting (FBS)

Investigation	Observed Value	Biological Reference Interval

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TEST REPORT

Glucose Fasting Method:Hexokinase	90	Normal: <100 mg/dL Impaired FG: 100-125 mg/dL Diabetes mellitus: >/=126 mg/dL

Interpretation: It measures the Glucose levels in the blood with a prior fasting of 9-12 hours. The test helps screen a symptomatic/ asymptomatic person who is at risk for Diabetes. It is also used for regular monitoring of glucose levels in people with Diabetes.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2022

Glucose Post Prandial (PPBS)

Investigation	Observed Value	Biological Reference Interval
Glucose Post Prandial Method:Hexokinase	96	Normal : <140 mg/dL Impaired PG: 140-199 mg/dL Diabetes mellitus: >/=200 mg/dL

Interpretation: This test measures the blood sugar levels 2 hours after a normal meal. Abnormally high blood sugars 2 hours after a meal reflect that the body is not producing sufficient insulin which is indicative of Diabetes.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2022

Glycosylated Hemoglobin (HbA1C)

Investigation	Observed Value	Biological Reference Interval
Glycosylated Hemoglobin (HbA1c) Method:High-Performance Liquid Chromatography	5.5	Non-diabetic: <= 5.6 % Pre-diabetic: 5.7 - 6.4 % Diabetic: >= 6.5 %
Estimated Average Glucose (eAG) Method:Calculated	111	mg/dL

Interpretation:

It is an index of long-term blood glucose concentrations and a measure of the risk for developing microvascular complications in patients with diabetes. Absolute risks of retinopathy and nephropathy are directly proportional to the mean HbA1c concentration. In persons without diabetes, HbA1c is directly related to risk of cardiovascular disease.

1) Low glycated haemoglobin (below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia (especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.

2) Interference of Hemoglobinopathies in HbA1c estimatiion:
A. For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
B. Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status
C. Heterozygous state detected (D10 is corrected for HbS and HbC trait).
3) In known diabetic patients, HbA1c can be considered as a tool for monitoring the glycemic control.
Excellent Control - 6 to 7 %,
Fair to Good Control - 7 to 8 %,
Unsatisfactory Control - 8 to 10 %
and Poor Control - More than 10 %.
Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2022.

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TEST REPORT

High Sensitive C reactive protein (HS-CRP)

Investigation	Observed value	Biological reference Range
H S CRP Method:Immunoturbidimetric	**2.29**	<1.0 mg/L **Note:** Biological Reference Ranges are changed due to change in method of testing.
Note	Kindly correlate clinically	

Interpretation:

Increased hs-CRP levels can indicate an increased risk for coronary heart disease in asymptomatic patients.

According to Center for Disease Control & American Heart Association (CDC & AHA), cardiovascular risk assessment guidelines for hs-CRP are as follows:-

- <1.0 mg/L: Low Risk
- 1.0 - 3.0 mg/L: Average Risk
- >3.0 mg/L: High Risk
- >10.0 mg/L: Possibly due to non-cardiovascular inflammation.

Disclaminer: Persistant unexplained elevation of HSCRP >10.0 mg/L should be evaluated for non-cardiovascular etiologies such as infections, active arthritis or concurrent illness.

Vitamin B12 (Cyanocobalamin)

Investigation	Observed Value	Biological Reference Interval
Vitamin B12 (Cyanocobalamin) ,Serum Method:ECLIA	376	197-771 pg/mL **Note:** Biological Reference Ranges are changed due to change in method of testing.

Interpretation:
1.Vitamin B12 is essential in DNA synthesis,haematopoiesis and CNS integrity.
2.Measurement of vitamin B12 is intended to identify and monitor vitamin B12 deficiency. This can arise from the following; (1) defect in the secretion of Intrinsic Factor, resulting in inadequate absorption from food (pernicious anemia); (2) gastrectomy and malabsorption due to surgical resection; and (3) a variety of bacterial or inflammatory diseases affecting the small intestine.(4) Decreased dietary intake.
3.Reduced concentrations of vitamin B12 may indicate the presence of vitamin dependent anemia.
4.Elevated concentrations of vitamin B12 have been associated with pregnancy, the use of oral contraceptives and multivitamins and in myeloproliferative diseases, such as chronic granulocytic leukemia and myelomonocytic leukemia. An elevated concentration of vitamin B12 is not known to cause clinical problems.

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Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Iron with TIBC

Investigation	Observed Value	Biological Reference Interval
Iron Method:Ferrozine-no deproteinization	**23.9**	33-193 µg/dL
Total Iron Binding Capacity (TIBC) Method:Calculated	443	168-585 µg/dL
Transferrin Saturation Index Method:Calculated	**5.4**	15-50 %
Note	Kindly correlate clinically	

Interpretation: Iron is a nutrient essential for several functions in the body including production of RBC's and proteins .Serum Iron concentrations are decreased in Iron deficiency Anemia and chronic inflammatory disorders. Elevated serum iron levels occur in iron-loading disorders, aplastic anemia, iron poisoning, etc.
TIBC measures the blood's capacity to bind iron with transferrin. It measures the maximum amount of Iron blood can carry. TIBC is increased in iron deficiency anemia and pregnancy .TIBC is decreased in anemia of chronic inflammation.

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TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Iron

Investigation	Observed Value	Biological Reference Interval
Iron Method:Ferrozine-no deproteinization	**24**	33-193 µg/dL
Note	Kindly correlate clinically	

Interpretation: Iron is a nutrient essential for several functions in the body including production of RBC's and proteins .Serum Iron concentrations are decreased in Iron deficiency Anemia and chronic inflammatory disorders. Elevated serum iron levels occur in iron-loading disorders, aplastic anemia, iron poisoning, etc.

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DEPARTMENT OF CLINICAL CHEMISTRY I

Lipid Profile

Investigation	Observed Value	Biological Reference Interval
Total Cholesterol Method:Cholesterol Oxidase	217	Desirable: <200 mg/dL Borderline: 200-239 mg/dL High: >/=240 mg/dL
HDL Cholesterol Method:Direct Measurement	**38**	Low: <40 mg/dL High: >/=60 mg/dL
VLDL Cholesterol Method:Calculated	22.20	6.0-38.0 mg/dL
LDL Cholesterol Method:Calculated	156.8	Optimum: <100 mg/dL Near/above optimum: 100-129 mg/dL Borderline: 130-159 mg/dL High: 160-189 mg/dL Very high: >/=190 mg/dL
Triglycerides Method:Glycerol LPL/GK	111	Normal:<150 mg/dL Borderline: 150-199 mg/dL High: 200-499 mg/dL Very high: >/=500 mg/dL
Chol/HDL Ratio Method:Calculated	**5.71**	Low Risk: 3.3-4.4 Average Risk: 4.5-7.1 Moderate Risk: 7.2-11.0
LDL Cholesterol/HDL Ratio Method:Calculated	**4.13**	Desirable: 0.5-3.0 Borderline Risk: 3.0-6.0 High Risk: >6.0
Non HDL Cholesterol Method:Calculated	**179**	<130 mg/dL
Note	Kindly correlate clinically	

Interpretation: Lipids are fats and fat-like substances which are important constituents of cells and are rich sources of energy. A lipid profile typically includes total cholesterol, high density lipoproteins (HDL), low density lipoprotein (LDL), chylomicrons, triglycerides, very low density lipoproteins (VLDL), Cholesterol/HDL ratio .The lipid profile is used to assess the risk of developing a heart disease and to monitor its treatment. The results of the lipid profile are evaluated along with other known risk factors associated with heart disease to plan and monitor treatment. Treatment options require clinical correlation.

Reference: Third Report of the National Cholesterol Education program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III), JAMA 2001.

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TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Liver Function Test (LFT)

Investigation	Observed Value	Biological Reference Interval
Total Bilirubin. Method:Diazo Method	0.46	<1.2 mg/dL
Direct Bilirubin. Method:Diazo Method	0.22	<0.30 mg/dL
Indirect Bilirubin. Method:Calculated	0.24	<0.9 mg/dL
Alanine Aminotransferase ,(ALT/SGPT) Method:UV wtihout P5P	13	<34 U/L
Aspartate Aminotransferase,(AST/SGOT) Method:UV wtihout P5P	18	<31 U/L
ALP (Alkaline Phosphatase). Method:PNPP-AMP Buffer	58	35-104 U/L
Gamma GT. Method:GCNA	12	6-42 U/L
Total Protein. Method:Biuret	7.1	6.6-8.7 g/dL
Albumin. Method:Bromocresol Green (BCG)	4.3	3.5-5.2 g/dL
Globulin. Method:Calculated	2.80	1.8-3.8 g/dL
A/GRatio. Method:Calculated	1.54	0.8-2.0
AST/ALT Ratio Method:Calculated	**1.38**	<1.00

Interpretation: Liver functions tests help to identify liver disease, its severity, and its type. Generally these tests are performed in combination, are abnormal in liver disease, and the pattern of abnormality is indicative of the nature of liver disease. An isolated abnormality of a single liver function test usually means a non-hepatic cause. If several liver function tests are simultaneously abnormal, then hepatic etiology is likely.

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Name	: **MRS.YAMUNA KALYANI NEPPALA**	TID/SID	: UMR3504977/ 30363136
Age / Gender	: 36 Years / Female	Registered on	: 20-Sep-2025 / 07:58 AM
Ref.By	: -	Collected on	: 20-Sep-2025 / 09:04 AM
Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 14:30 PM
Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Phosphorus, Serum

Investigation	Observed Value	Biological Reference Interval
Phosphorus Method:Phosphomolybdate	3.7	2.5-4.5 mg/dL

Intepretation: Phosphorus is a vital component of bones & teeth, several lipoproteins and nucleoproteins. Phosphorus levels are important to diagnose and monitor treatment of various conditions that cause calcium and phosphorus imbalances. Phosphorus levels in urine samples are vital to monitor its elimination by the kidneys.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Afreen Anwar
Consultant Biochemist

tenet® DIAGNOSTICS
It's Good to Know

MC-2977

PLEASE SCAN QR CODE
TO VERIFY THE REPORT ONLINE

Name	: **MRS.YAMUNA KALYANI NEPPALA**	TID/SID	: UMR3504977/ 30363136
Age / Gender	: 36 Years / Female	Registered on	: 20-Sep-2025 / 07:58 AM
Ref.By	: -	Collected on	: 20-Sep-2025 / 09:04 AM
Req.No	: 25GCBH0007003	Reported on	: 20-Sep-2025 / 14:06 PM
Sample Type	: Serum	Reference	: HCL Avitas Pvt Ltd

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Uric Acid, Serum

Investigation	Observed Value	Biological Reference Interval
Uric Acid. Method:Uricase	3.4	2.4-5.7 mg/dL

Interpretation

It is the major product of purine catabolism. Hyperuricemia can result due to increased formation or decreased excretion of uric acid which can be due to several causes like metabolic disorders, psoriasis, tissue hypoxia, pre-eclampsia, alcohol, lead poisoning, acute or chronic kidney disease, etc. Hypouricemia may be seen in severe hepato cellular disease and defective renal tubular reabsorption of uric acid.

* Sample processed at National Reference Laboratory,Tenet Diagnostics,54, Kineta Towers, Journalist Colony, Banjara Hills

--- End Of Report ---

Dr Afreen Anwar
Consultant Biochemist

Yamuna
ID: 7003

20.09.2025 9:12:26
TENET DIAGNOSTICS
GACHIBOWLI
HYDERABAD

Location:
Room:
Order Number:
Indication:
Medication 1:
Medication 2:
Medication 3:

77 bpm
-- / -- mmHg

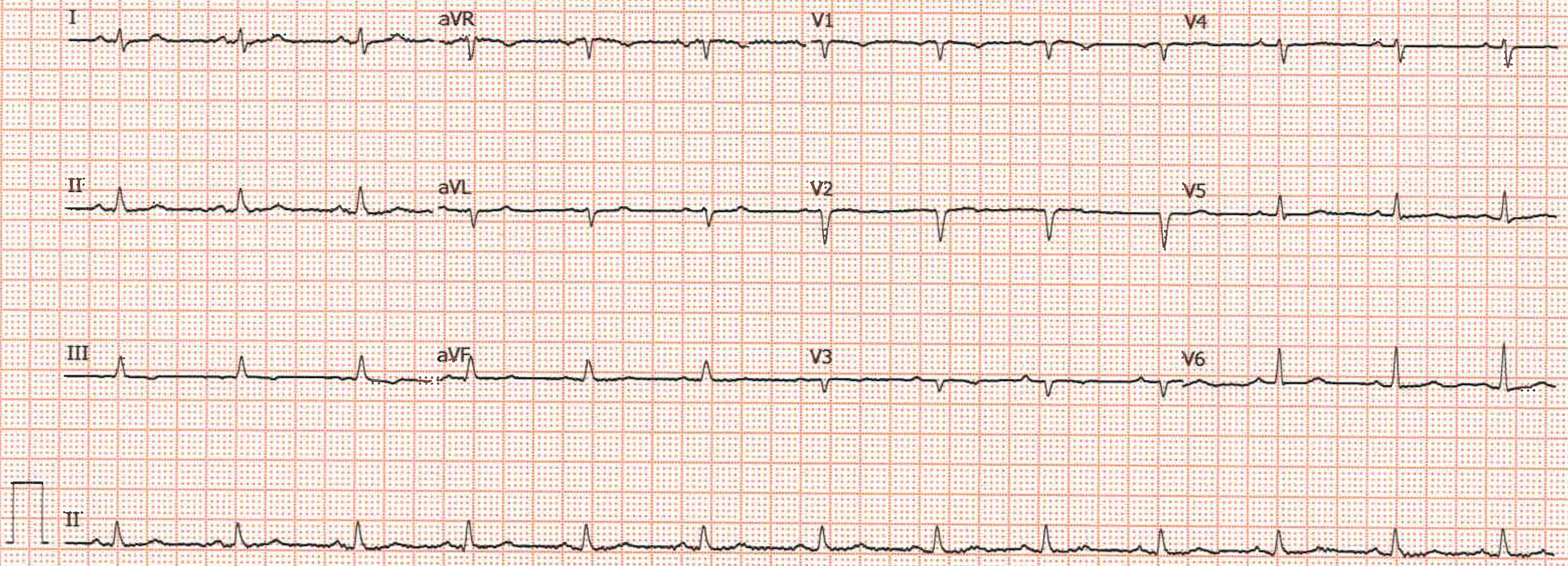
QRS :	68 ms	
QT / QTcBaz :	364 / 411 ms	
PR :	154 ms	
P :	102 ms	
RR / PP :	774 / 779 ms	
P / QRS / T :	30 / 84 / 43 degrees	

Normal sinus rhythm
Low voltage QRS
Septal infarct , age undetermined
Abnormal ECG

• NSR
• Low QRS Complexes

Technician:
Ordering Ph:
Referring Ph:
Attending Ph:

Dr. Y. Rohit Rao
MBBS
General Physician



GE MAC2000 1.1 12SL™ v241 25 mm/s 10 mm/mV ADS 0.56-20 Hz Unconfirmed 4x2.5x3_25_R1 1/1

tenet® DIAGNOSTICS
It's Good to Know

PLEASE SCAN QR CODE

Name	: **Mrs . YAMUNA KALYANI NEPPALA**	TID	: UMR3504977
Age/Gender	: 36 Years/Female	Registered On	: 20-Sep-2025 07:58 AM
Ref By	:	Reported On	: 20-Sep-2025 10:43 AM
Reg.No	: 25GCBH0007003	Reference	: HCL Avitas Pvt Ltd

DEPARTMENT OF ULTRASOUND

Sonomammography Bilateral

Findings :

Both the breasts show normal echopattern.

No evidence of focal solid / cystic areas.

No evidence of ductal dilatation.

No evidence of axillary lymphadenopathy on both sides.

IMPRESSION:

*** No significant sonographic abnormality - BI-RADS Category - 1.**

BIRADS CLASSIFICATION

CATEGORY	RESULT
0	Assessment incomplete. Need additional imaging evaluation
1	Negative. Routine screening in 1 year recommended.
2	Benign finding. Routine screening in 1 year recommended.
3	Probably benign finding. Short interval followup suggested.
4	Suspicious. Biopsy should be considered.
5	Highly suggestive of malignancy. Appropriate action should be taken.

- Suggested clinical correlation and follow up.

*** End Of Report ***

Typed By: chamanthi

K. Apoorva

Dr. Apoorva K
Consultant Radiologist

PLEASE SCAN QR CODE

Name	**: Mrs . YAMUNA KALYANI NEPPALA**	TID	: UMR3504977
Age/Gender	: 36 Years/Female	Registered On	: 20-Sep-2025 07:58 AM
Ref By	:	Reported On	: 20-Sep-2025 12:36 PM
Reg.No	: 25GCBH0007003	Reference	: HCL Avitas Pvt Ltd

DEPARTMENT OF X-RAY
X-Ray Chest PA View

Clinical History : Health check up

Lung fields appear normal.

Cardiac size is within normal limits.

Aorta and pulmonary vasculature is normal.

Bilateral domes of diaphragm and costophrenic angles are normal.

Visualised bones and soft tissues appear normal.

IMPRESSION:

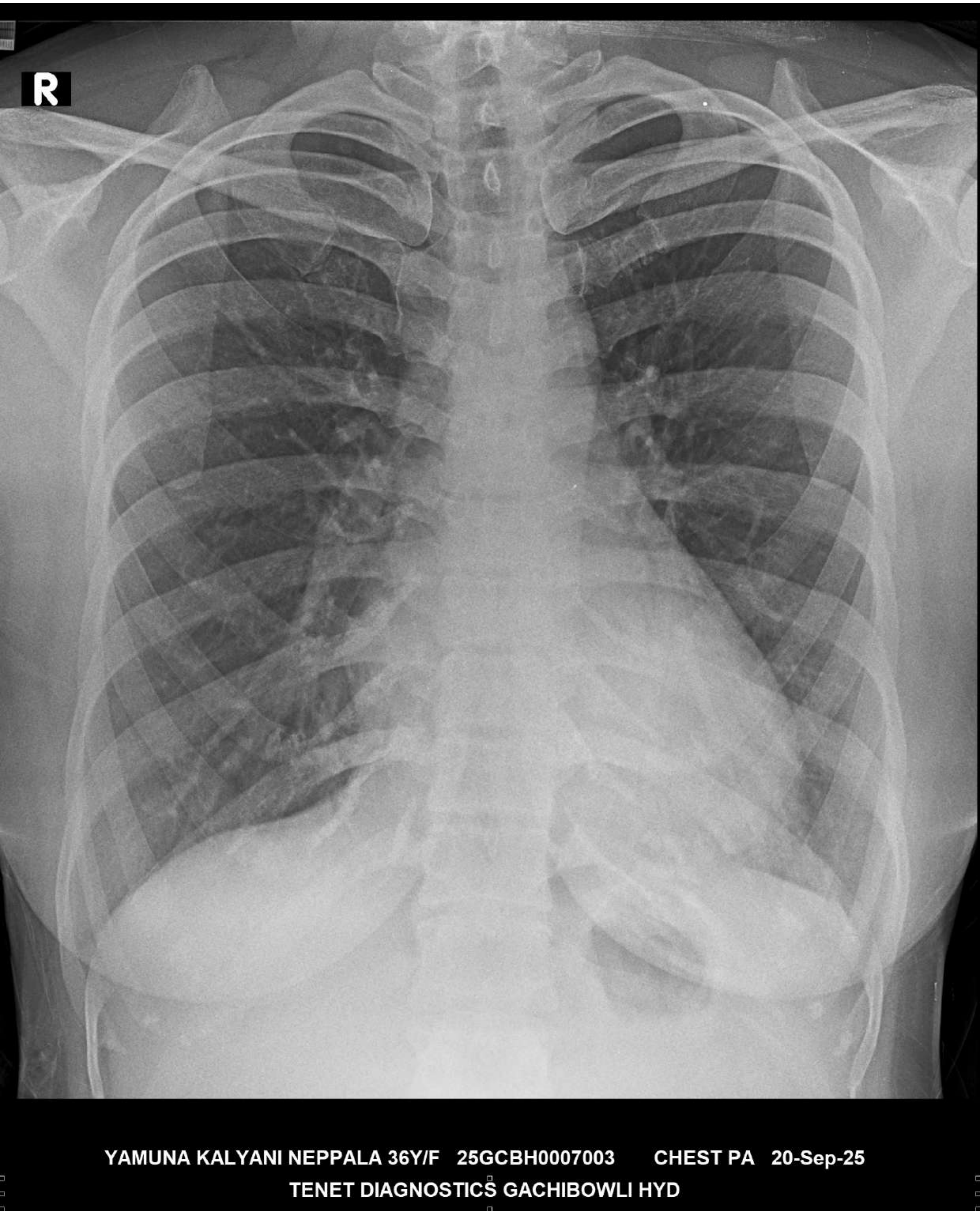
*** Normal study.**

Suggested clinical correlation and follow up.

*** End Of Report ***

Typed By: Parveensulthana

Dr.S Sireesha
Consultant Radiologist


R
YAMUNA KALYANI NEPPALA 36Y/F 25GCBH0007003 CHEST PA 20-Sep-25
TENET DIAGNOSTICS GACHIBOWLI HYD